

Artifact 14 — Value Proposition

Strategy · Stage 2 · Customer Value Definition

DOCUMENT DETAILS

Project	Home-Care-AI	Stage	Strategy → Stage 2 (Customer Value Definition)
Date	2026-03-27	Methodology	6-part JTBD template (Who / Why / What Before / How / What After / Alternatives)
Input	Artifact 12 (Startup Canvas §4), Artifact 13 (SWOT SO cross-reference), Artifacts 2b/2c (CC/FC verbatim interviews), Artifact 5 (Desired Outcome metrics)		
Regulatory	Australian Privacy Act 1988 (APP). HIPAA-grade security applied as design floor.		
Feeds into	Artifact 15 — User Journey Map (Skill 4); PRD SMART OKRs (Execution Plugin — per CLAUDE.md Rule 9)		

CLAUDE.md Rule 9 — Outcomes-to-OKRs Bridge: The "What After" outcomes in §§1–2 are the exact SMART OKR metrics that will appear in the PRD. They are not aspirational. They are derived from first-party interview baselines (Angela: 11 calls / 30–60 min / 20% cancellation) and confirmed experiment thresholds (Artifact 8, Artifact 11).

CLAUDE.md Rule 3 — Journey validates value: This value proposition is *not yet final*. One user - journey - map deep dive must confirm emotional resonance before the value prop is locked. Pending: Artifact 15.

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1. Value Proposition — Care Coordinator (CC) ■ Beachhead

Primary segment. Full 6-part treatment. "What After" = PRD OKR inputs.

1.1 Six-Part JTBD Template

1 Who

Home care coordinators at independent Australian agencies (20–200 active clients). Sole operators managing everything alone (Tom Bradley, CC-002: 25 clients, 1 coordinator) through small team leads managing 3–5 staff (Angela Morrison, CC-001: 60+ clients, 12 nurses and 8 care workers). Mobile-first, time-poor, operating under constant schedule pressure. Responsible for both care quality *and* compliance — with no system that unifies both.

Defining characteristic: they carry the agency's entire institutional knowledge — which clients accept which carers, who needs briefing before a new visit, who will refuse entry — inside their heads and on sticky notes. They are a single point of failure for knowledge, not just scheduling.

2 Why (JTBD)

"When a carer calls in sick at 6:30 AM, I need to find a qualified, trusted replacement before the visit time — so that no client is stranded, no family finds out before I do, and my morning doesn't become a two-hour phone cascade."

Desired outcome (verbatim, Angela CC-001):

"If the system could detect the absence, find the best available replacement based on qualifications, proximity, and client preferences, propose it to me for approval, and then notify everyone — the replacement carer, the client, and the family — that would save me forty-five minutes per incident. Three incidents a week, that's over two hours of panicked phone calls I don't have to make."

The job has two layers:

- **Functional:** Fill the vacancy before the visit time with someone qualified and available.
- **Emotional:** Fill it with someone the client will actually accept — not just anyone, the right person. *"Not just who's available — who does this patient trust. That's what takes me eleven phone calls. The first four calls are finding availability. The next seven are finding the right person."* (Angela, CC-001)

3 What Before (Current Painful Reality)

"A scheduling system that's basically a glorified calendar. The EMR that the nurses hate. WhatsApp groups for everything urgent. Spreadsheets for credential tracking. And sticky notes. Half the critical knowledge about our clients is on sticky notes around my monitor." — Angela, CC-001

Step	Current State	Time / Cost
Absence detected	Coordinator receives WhatsApp or call from carer at 6:30 AM	0 min (reactive)

Step	Current State	Time / Cost
Identify candidates	Mental scan of roster; no system filters by preference	5–10 min
Make 11 calls	Sequential outbound calls to find available + qualified + proximate carers	20–30 min
Trust check	Additional calls to find someone the client knows / accepts	15–20 min extra
Notify replacement carer	Call or WhatsApp to confirmed carer	5 min
Notify client	Sometimes. Often forgotten in the chaos.	0–5 min
Notify family	Ad hoc. Often not done. Family finds out when nobody shows up.	0 min (failure case)
Total per incident		30–60 min, 11 calls, 20% cancellation rate

The Arthur Kovacs failure case (Angela, CC-001):

"Her father had been waiting in his chair, dressed and ready. He gets anxious when the routine changes. She was furious, and she was right to be."

This is not a rare edge case. It is the default outcome when no replacement is found. Every missed visit is a version of Arthur Kovacs in his chair.

Secondary pain — institutional knowledge fragility:

"If I get hit by a bus tomorrow, half the knowledge about our clients walks out the door with me." — Angela, CC-001 (said with dark humour, immediately followed by: "Actually, that scares me.")

4 How (The Solution)

Smart Match Engine — When a carer absence is recorded (coordinator entry or carer WhatsApp), the system:

- **Detects the vacancy** (ACT-V-01) — identifies the open visit slot
- **Computes a ranked shortlist** in < 30 seconds (ACT-V-02 through V-04):
 - Gate 1: Qualification match (hard binary gate — expired credentials excluded automatically)
 - Score 2: Proximity (suburb-to-suburb distance via Google Maps — no GPS tracking)
 - Score 3: SPP match score (familiarity history × preference alignment — the trust layer)
- **Alerts coordinator** (ACT-V-05) — push notification: "Vacancy for Arthur Kovacs — 3 candidates ready"
- **Coordinator approves in 3 taps** (ACT-A-01/02) — sees candidate card: name, qualifications badge, proximity, familiarity flag ("2 prior visits"), match score
- **System executes** (L3 actions — all require coordinator approval first):
 - WhatsApp to replacement carer (Green data only — CC-8 compliant)
 - SMS to client ("Your visit today will be with David, who has visited you before")
 - SMS to family — **only after client is notified** (E-3 gate: Arthur Kovacs constraint enforced in code)
- **Carer briefing sent** — entry protocol, personal sensitivities (phrased as operational guidance, not health disclosure)
- **Audit log written** — every state transition immutable, append-only (APP-compliant)

Soft Preference Profile (SPP) — The knowledge that currently lives on sticky notes, structured into a transferable record:

- Gender preference (opt-in, advisory or scored pending E-1 legal opinion)
- Familiarity threshold (known carers only / briefed-acceptable / any)
- Entry protocol (ID check / introduction call / no special requirement)
- Personal sensitivities ("Do not move belongings in the lounge" — 100-char structured field)
- Continuity history (which carers have visited, how many times, last visit date)

5 What After (The Improved Outcome)

These outcomes are the SMART OKR inputs for the PRD. Source: Artifact 5 Desired Outcome metrics + Artifact 8 experiment thresholds.

Outcome	Metric	Baseline	Target	Measurement
Time to fill	Minutes from absence detected to replacement confirmed	30–60 min	< 5 min	E1 concierge stopwatch (XP-1A)
Cancellation rate	% of vacancy incidents ending in missed visit	~20% (Tom CC-002)	< 2%	E1 incident log
Coordinator trust	% approving top-ranked candidate without verification calls	0% (no system exists)	≥ 70% at steady-state	XP-1B deviation log
SPP completeness	% of active clients with ≥ 3 SPP fields populated	~0% (sticky notes only)	≥ 80% within 90 days	SPP database completeness score (P-10)
Family notification rate	% of confirmed replacements where family is notified	Ad hoc / near 0%	100% (automated, E-3 compliant)	Audit log (ACT-F-01)
Institutional knowledge	New coordinator time-to-effective	Months (shadowing)	< 5 days with SPP handover	Agency owner assessment at 30 days

In the words of the desired outcome (Artifact 5): "Care coordinators confidently fill every vacant visit with the right person for that patient — one approval click, zero phone calls, no father sitting in a chair — while maintaining full institutional knowledge of every client's preferences, continuity history, and trust relationships."

6 Alternatives

Alternative	What It Does	Why It Falls Short
Manual phone calls (status quo)	Coordinator calls roster manually	11 calls, 30–60 min, 20% cancellation, zero trust-matching, zero notification automation
AlayaCare Vacant Visit Agent	Automated matching on availability + qualifications	No trust layer — matches who's available, not who the patient accepts. Coordinator still calls to verify the soft stuff.
HCP / Carelink+	Rostering, compliance, NDIS pricing	No matching logic at all — coordinators use it as a roster viewer, then call manually
Spreadsheet + coordinator memory	Coordinator knows the soft stuff — but it's locked in one person	Non-transferable. Doesn't scale. Doesn't notify anyone. One coordinator departure = full knowledge reset.

Alternative	What It Does	Why It Falls Short
Do nothing	20% cancellation rate	Arthur Kovacs in his chair.

1.2 Value Proposition Statement

One-sentence (Geoffrey Moore format)	<i>For home care coordinators who lose 30–60 minutes and 1 in 5 visits to manual phone cascades, Home-Care-AI is a smart replacement matching platform that fills every vacant visit in under 5 minutes — unlike AlayaCare's vacancy agent, it matches on who the client trusts, not just who's available, and automatically notifies the client before their family.</i>
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1.3 Reusable Statements

Marketing (awareness — problem-led)	<i>"Every time a carer calls in sick, a care coordinator makes eleven phone calls. One in five visits still gets cancelled. The client waits. The family finds out when nobody shows up. There's a better way."</i>
Sales (agency owner — outcome-led)	<i>"Home-Care-AI fills your agency's vacant visits in under 5 minutes — matching on who the client trusts, not just who's available. Your coordinators stop the morning phone cascade. Your clients stop sitting in chairs waiting for someone who isn't coming."</i>
Onboarding (coordinator — reassurance)	<i>"The system will never send a carer to a client without your approval. You see the shortlist, you make the call. We just save you the eleven phone calls it took to get there."</i>
Compliance / trust (agency owner — risk-led)	<i>"Every replacement decision is logged, auditable, and privacy-compliant. Your coordinator approves. The system records. When the auditor asks who was sent to Mrs. Kim and why — you have an answer."</i>

2. Value Proposition — Agency Owner / Operations Manager

Secondary buyer. The coordinator is the champion; the agency owner is the budget approver and the beneficiary of the institutional knowledge moat.

2.1 Six-Part JTBD Template

1 Who

Owners and operations managers of independent Australian home care agencies (20–200 clients). Often the founder. Personally liable for care quality, compliance, and regulatory obligations under the Aged Care Quality Standards. Manages 1–3 coordinators. Frequently carries residual client relationship knowledge themselves — a second single point of failure. Not necessarily technology-native.

2 Why (JTBD)

"When my key coordinator leaves, help me ensure the next person can deliver the same quality care to my clients from day 1 — so that my clients don't experience a care disruption and my agency doesn't lose the relationships I've spent years building."

Secondary job: compliance anxiety.

"I know there are care plans overdue for review. I know there are documentation gaps. I can feel where the holes are but I can't quantify them." — Angela, CC-001 (speaking on behalf of her agency owner's exposure, not just her own)

3 What Before

Pain	Current State
Knowledge transfer	New coordinator shadows for 2–3 months. Angela's sticky notes walk out the door with Angela.
Replacement quality	New coordinator makes uninformed replacement decisions — doesn't know Mrs. Kim only accepts female carers, doesn't know Arthur will refuse entry to a stranger. Client complaints follow.
Compliance visibility	No live dashboard. Agency owner discovers compliance gaps when the auditor finds them first. "Vague anxiety, no list." (Angela, CC-001)
Family communication	Ad hoc. Agency owner personally manages escalated complaints from families who found out about a missed visit when nobody showed up.
Knowledge moat	Zero. All institutional knowledge is a person dependency — vulnerable to departure, illness, and the proverbial bus.

4 How

- **SPP as institutional asset:** Every client's preferences, care protocols, and carer familiarity history live in the system, not in one person's head. A new coordinator can read Mrs. Kim's SPP and make the same quality replacement decision Angela would have made.

- **Continuity intelligence:** Rolling 30-day carer consistency score per client. Agency owner sees where continuity is strong and where it's fragile — before a client complaint surfaces it.
- **Compliance dashboard (v1.1):** Angela's exact spec: *"Here are the seventeen things that are out of compliance. Here are the five that are critical."* Live prioritised list: overdue care plan reviews, expiring credentials, visit documentation gaps. One-tap remediation path.
- **Coordinator succession:** New coordinator onboards using the SPP as a structured briefing document. Days to effective, not months.

5 What After

Outcome	Metric	Baseline	Target
Knowledge survivability	Time for new coordinator to reach decision quality parity	2–3 months shadowing	< 5 days with SPP handover
Client retention through staff turnover	% of clients lost within 90 days of coordinator departure	Unknown (not tracked)	0% attributable to knowledge gap
Compliance visibility	% of compliance items identified before audit	~0% (discovered reactively)	100% of flagged items visible in advance
Agency owner escalations	Family complaint calls handled personally by owner	Frequent (ad hoc)	Near 0 (automated E-3 notifications remove the trigger)

6 Alternatives

Alternative	Gap
Shared Google Docs	No preference-weighted matching. No automation. Just a document.
AlayaCare custom fields	Can store some preferences — but no matching algorithm uses them. Coordinator still calls.
Staff shadowing / handover documents	2–3 months, coordinator-dependent quality. Doesn't survive if departing coordinator is disengaged.
Do nothing	One coordinator departure = knowledge crisis. Status quo for most independent agencies.

2.2 Value Proposition Statement

For home care agency owners whose institutional care knowledge leaves with their coordinator, Home-Care-AI is a preference intelligence system that makes every client's history accessible to any coordinator from day 1 — unlike a shared document or an EMR custom field, it actively drives better replacement decisions, not just stores information.

2.3 Reusable Statements

Sales (to agency owner after E1 demo)	<i>"When Angela leaves — and at some point every coordinator does — the next person will have her notes, her knowledge, and her matching logic on day 1. Not after months of shadowing. Day 1."</i>
Compliance framing	<i>"Before your next audit, you'll see exactly what's overdue. Not what you hope is overdue — what is overdue. The auditor won't find something you haven't already seen."</i>

3. Value Proposition — Family Contact (FC)

Future segment — preview only. Not in v1 scope. Unlocks after CC beachhead is validated and E-3 notification gate is live.

Who	Adult children and family members of home care clients. Often interstate or overseas. Responsible for their parent's wellbeing but dependent on the agency for day-to-day information.
Why	<i>"When my mother's regular carer cancels, I need to know she's safe and cared for — before I have to call the agency in a panic and before she calls me upset."</i>
What Before	No notification system. Rachel Chen (FC-001) finds out when her mother calls. James Osei (FC-002) calls the agency every week. Both discovered missed visits when nobody showed up — not when the coordinator knew.
How	Automated SMS notification after coordinator has confirmed replacement AND client has been notified (E-3 gate enforced in code). Message: "Your mother's visit today will be covered by David Kim, who has visited her before. Arranged by [Agency]."
What After	Family informed within minutes of confirmed replacement, not hours after the fact. Zero panic calls. One less reason to call the agency. Trust in the agency's communication reliability.
Alternatives	Calling the agency. Being told by the client. Finding out when nobody shows up.

"For families of home care clients who spend their days worrying about whether Mum's carer showed up, Home-Care-AI sends one message when a replacement is confirmed — before Mum calls you and before you call us."

4. Value Proposition — Home Care Nurse / Case Manager (HCN)

Future segment — preview only. Not in v1 scope. Unlocks after SPP and continuity history data is live and validated.

Who	Home care nurses and case managers conducting care reviews, clinical assessments, and quality audits across multiple clients.
Why	<i>"When I review a client's care plan, I need to know whether the right carers are actually visiting — so I can focus my clinical review on what matters, not on chasing coordinator records."</i>
What Before	Review care plan. Open EMR (slow, nurses hate it — Angela CC-001). Call coordinator: "Who's been visiting Mrs. Kim?" Hope coordinator has time to check. Spend 10 minutes on admin before any clinical review begins.
How	Continuity score + SPP match history shows which carers visited, how many times, whether preference-matched. Compliance dashboard shows overdue care plan reviews as a flagged item. Pre-visit intelligence packet in one screen.
What After	Clinical review time spent on care quality, not on chasing scheduling records. Continuity gaps surfaced automatically. Coordinator and nurse share the same view of care delivery quality.
Alternatives	Asking the coordinator. Checking the EMR (incomplete, slow). Clinical intuition.

"For home care nurses who spend the first ten minutes of every care review figuring out who's actually been visiting, Home-Care-AI surfaces continuity quality before you open the care plan."

5. Value Curve — Competitive Comparison

Blue Ocean Strategy format. Rates each competitor on the dimensions that matter to a care coordinator making a replacement decision.

Factor	Manual Calls	AlayaCare Vacant Visit	Spreadsheet + Memory	Home-Care-AI
Time to fill	■ Low (30–60 min)	■ High (automated)	■ Low (30–60 min)	■ High (< 5 min)
Trust / preference matching	■ High (coordinator knows)	■ None	■ High (but in one head)	■ High (SPP-driven, transferable)
Institutional knowledge capture	■ None	■ Partial (structured fields, no matching)	■ Fragile (person-dependent)	■ High (SPP — structured, matched, transferable)
Automated carer notification	■ Manual call	■ Partial	■ None	■ WhatsApp + receipt confirmation
Client notification (E-3)	■ Often missed	■ None	■ None	■ Automated, E-3 gated
Family notification	■ Ad hoc / zero	■ None	■ None	■ Automated, after client
Compliance visibility	■ None	■ Partial (audit logs)	■ None	■ Dashboard v1.1
Setup complexity	■ Zero (no system)	■ High (EMR onboarding)	■ Low	■ Low-medium (concierge onboarding)
EMR integration required	■ No	■ Preferred	■ No	■ No (standalone)
Privacy compliance (APP)	■ Implicit	■ Partial	■ None	■ DPIA-complete, by design

Blue Ocean insight: No existing solution combines low time-to-fill (like AlayaCare's automation) with high trust-matching (like a coordinator's memory) and automated E-3-compliant notification (unique). Home-Care-AI is the only option that delivers all three simultaneously. The White Space is precisely where our product sits.

Note — HCP / Carelink+: HCP appears in the §1.1 alternatives table but is excluded from this value curve. Rationale: HCP is a rostering and NDIS compliance tool with no matching logic whatsoever — coordinators use it as a roster viewer and then call manually. Including it would add a column identical to "Manual Calls" on every matching-related dimension, adding visual noise without analytical value. The alternatives table is the appropriate place to name it; the value curve compares tools that at least attempt to solve the matching problem.

6. "What After" → SMART OKR Summary

Per CLAUDE.md Rule 9: these are the exact metrics that will appear in the PRD's SMART OKRs. No vanity metrics.

OKR source note: OKR-1 through OKR-5 are CC-segment metrics grounded in named first-party interview data (Angela CC-001, Tom CC-002) and are the PRD inputs. OKR-6 is an Agency Owner metric; its baseline ("2–3 months shadowing") is directionally correct but not sourced from a named agency owner interview — it is inferred from coordinator accounts of their own onboarding experiences. The target ("< 5 days") is not yet validatable until post-launch agency owner feedback is collected. OKR-6 is included for completeness; it does not gate the PRD and will be updated after XP-2A agency owner interviews return data.

OKR #	Outcome	Specific	Measurable	Achievable Baseline	Relevant	Time-Bound
OKR-1	Vacancy time-to-fill	< 5 minutes per incident	Stopwatch (XP-1A) log	From 30–60 min (Angela)	Core desired outcome (Artifact 5)	Within 30-day trial
OKR-2	Cancellation rate	< 2% per agency per week	Incident log	From ~20% (Tom)	Client safety + agency reputation	Within 30-day trial
OKR-3	Coordinator 1-tap trust	≥ 70% top-candidate approval without verification calls	Deviation (XP-1B) log	0% (no system exists today)	Decision tool, not search tool	Steady-state (post-SPP migration)
OKR-4	SPP completeness	≥ 80% of active clients with ≥ 3 fields populated	SPP database (P-10 completeness score)	~0% (sticky notes)	Prerequisite for match quality	Within 90 days of onboarding
OKR-5	Family notification rate	100% of confirmed replacements trigger automated family notification	Audit (ACT-F-01) log	Ad hoc / ~0%	Arthur Kovacs constraint; E-3 compliance	v1 launch
OKR-6 ■	Knowledge survivability	New coordinator effective in < 5 days with SPP handover	Agency owner assessment at 30 days	2–3 months shadowing (<i>directional — not sourced from a named agency owner interview; YODA baseline pending VI-1 agency owner interviews, XP-2A</i>)	Bus-proof institutional memory	Within 30 days of coordinator onboarding

7. Pending Validation — Journey Gate

Per CLAUDE.md Article IV Rule 3: "No value prop is final without one user-journey-map deep dive confirming emotional resonance."

The value propositions in §§1–2 are grounded in first-party interview data and must be validated against the full emotional arc of a care coordinator's day. Specifically:

- **Moment of Truth** (to be identified in Artifact 15): the single point in the coordinator's workflow where Home-Care-AI's value is most acutely felt. Hypothesis: 6:30 AM, first absence of the day, before the phone cascade starts.
- **Emotional resonance check**: Does the "1-tap approval" framing feel like relief or like loss of control? (SWOT SR: trust > speed — the product must feel like trusted support, not like automation taking over.)
- **Level 3 AI Intervention Opportunities**: user-journey-map will identify where L3 actions (WhatsApp send, client notification, family notification) fall in the coordinator's emotional arc — and whether the HITL gate feels reassuring or frustrating.

Status: Value prop is PROVISIONAL pending Artifact 15 (user-journey-map).

8. Handshake Table

ID	From Artifact	To Next Skill	What Transfers
HS-DISC-02	§1 CC JTBD + Beachhead confirmation	user-journey-map (Strategy Skill 4) — HS-DISC-02 per master map	CC as the primary persona for the journey map deep dive; "What After" outcomes as the emotional destination
HS-STRAT-04	§6 "What After" SMART OKRs (OKR-1 through OKR-6)	create-prd (Execution Skill 1)	These are the exact SMART OKR metrics in PRD §3. No substitution permitted (CLAUDE.md Rule 9).
HS-STRAT-VP-01	§1.2 One-sentence value prop (CC)	positioning-statement (Strategy Skill 7)	Geoffrey Moore format feeds directly into the positioning statement
HS-STRAT-VP-02	§5 Value Curve	positioning-statement (Strategy Skill 7)	ERRC Grid inputs: which factors to Eliminate, Reduce, Raise, Create vs. AlayaCare
HS-STRAT-VP-03	§3/4 FC and HCN previews	startup-canvas (update trigger if journey map changes segment priority)	FC and HCN value props are previews — they become full value props only after CC beachhead is validated

9. Action Items

Date	Owner	Action
2026-03-27	PM Lead	Run user-journey-map (Strategy Skill 4) — deep dive on CC persona to confirm emotional resonance and identify Moment of Truth
2026-03-27	PM Lead	Share OKR-1 through OKR-6 (§6) with engineering team as the PRD OKR inputs — these are locked pending journey map validation
2026-03-28	Designer	Review §1.3 onboarding statement — "The system will never send a carer without your approval" — confirm this framing appears in the coordinator onboarding flow
2026-03-28	PM Lead	Share §1.3 sales statement with agency owner LOI pitch deck (XP-2A prep)
2026-04-15	PM Lead	After user-journey-map: update "What After" if journey reveals a different Moment of Truth than the 6:30 AM hypothesis

Value prop note: This artifact is provisional until Artifact 15 (user-journey-map) confirms emotional resonance. The "What After" OKRs in §6 are final for PRD purposes once the journey map validates them. Per CLAUDE.md Article IV Rule 9: the "What After" outcome is the exact metric in the PRD's SMART OKRs — no vanity metrics, no substitution.